جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ايمان جمعة منجد محمد
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** حلوان
رقم البطاقة : 28502210102583 **رقم التليفون :** 01129891750 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 21-02-1985 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ايمان جمعة منجد محمد
National ID: 28502210102583

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين

تم تغيير الموبايل من الموبايل الفيسجو
إلى فيريسيتن تحت الجلد



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY
INTERNATIONAL CANCER

Patient Name	: الأستاذه / ايمان جمعه منجد محمد	Request No.	: 13004552
Patient No.	: 28502210102583	Request Date	: 18/01/2026
Path No.	: 138/26	Verification Date	: 22/01/2026
Sex	: Female	Financial Category	: نفقة الدولة - جوستاف روسي
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: A case of left breast intermediate grade DCIS with microinvasion, Post-NACT.
Nature of Specimen: Left modified radical mastectomy.

Gross Examination:
* Left modified radical mastectomy specimen measured 22x18x6 cm, with covering skin ellipse measured 18x8 cm. Nipple & areola looked unremarkable grossly.
* Serialling showed a tumor bed site 9x7x3 cm, located 3 from the least deep margin.
* Axillary fat measured 8x7x2 cm, entangling 11 lymph nodes, the largest measured 2.5x2x1 cm.
* 15 Blocks:
1:9- T.B.S., 10- N&A., 11- Deep margin., 12:15- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic Examination:
* Histological type: No residual viable remnants of invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.
* Nipple & areola: Free.
* Deep margin: Free.
* Axillary lymph nodes: Eleven lymph nodes free of metastatic deposits (0/11).
* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: ايمان جمعه منجد **MR. :** 608	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 41-year-old premenopausal individual diagnosed with left breast invasive ductal carcinoma, ER-negative, PR 3/8, HER2 3+, cT2 N1, with a CT2 N1 staging and a left mastectomy performed in January 2025. The patient received neoadjuvant chemotherapy with TCHP and underwent left modified radical mastectomy (MRM) with a healed surgical scar and good range of motion. The multidisciplinary team (MDT) recommended adjuvant radiotherapy and a one-year course of Herceptin (trastuzumab) following the surgery.

The MDT meeting on 9 February 2026 confirmed the plan for Herceptin subcutaneous (SC) administration and a port for infusion. The patient achieved an estimated 68% efficacy (EF 68%) as of 14 December, and the plan was to continue with cycle 8 of PHESGO until the Herceptin course was completed.

On 16 April 2026, the pharmacy noted that cycle 9 of PHESGO was postponed until the drug became available, with an estimated efficacy of 69% as of 15 April.

On 13 May 2026, the pharmacy recorded a new MDT decision that the patient would receive Herceptin SC 600 mg for one year, starting with cycle 1 of Herceptin SC.

On 11 August 2026, the internal medicine note documented that the patient received Herceptin SC 600 mg for cycle 1, and cycles 2 and 3 were administered via port, with the treatment ending in April 2026. The patient has completed 3 cycles of Herceptin SC.

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / ايمان جمعه منجد محمد	Request No.	: 13004552
Patient No.	: 28502210102583	Request Date	: 18/01/2026
Path No.	: 138/26	Verification Date	: 22/01/2026
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- Deep margin: Free.
- Axillary lymph nodes: Eleven lymph nodes free of metastatic deposits (0/11).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مستشفى ...

مستشفى الأمير سلطان بن عبد العزيز للخدمات الطبية الملكية
مدينة الأمير سلطان الطبية

مدينة الأمير سلطان الطبية

MDT FORM 8/7/2025 G

Name	[illegible]	MRN	1311/25
Age	41	ID	[illegible]
Menopausal status	peri menopausal	Phone	[illegible]
NRS SCORE		Marital status	M
BMI	Average	Family history	(+ve) mother
CO morbidity	Lumbar disc Prolapse	REASON OF MDT	new case

Clinical Data

BX: Intermediate DCIS + Micro invasive focus

ER:	PR:	HER 2:	KI67:
0/8	3/8	+3	25%

Sonomammography: Lt breast UIQ mass 4 x 2-3 x 3 cm ? possible ductal extension

Mets Workup:

CT CAP → free

Bone scan → Requested

LN → -ve

R L

TNM

Recommendations:

[illegible] السعدي

MDT OUTCOME & PLAN

(1) Clips insert

(2) TCHP X 6 cycles (adjuvant Sc...)

(3) PSCS

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
[illegible]	Dr Hind 8/7/2025				

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الاستاذه / ايمان جمعه منجد محمد		
Patient No.	: 28502210102583	Request No.	: 13002199
Path No.	: 1131/25	Request Date	: 24/03/2025
Sex	: Female	Verification Date	: 14/04/2025
Age	: 40 Y	Financial Category	: اورام دار السلام - تغطية الدولة
Referreing Doc :		Department	: Out Patient
		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast 11 o'clock suspicious hypoechoic lesion.
Nature of Specimen: Tru-cut biopsy, collected at 23/3/2025.

Gross Examination:
- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for P63, Calponin, ER, PR, Her2, P63 clone (4A4), Calponin clone (EP798Y), ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
Histological type: Intermediate grade DCIS (Solid & cribriform), with microinvasive focus.
Perineural invasion: No.
Angioinvasion: No.
Microcalcifications: Undetected.
IPT (Done in microinvasive focus):
P63: Negative.
Calponin: Negative.
ER: No nuclear reaction in tumor cells (score 0/8).
PR: weak nuclear reaction in 5% of tumor cells (score 3/8).
Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
Ki67: 25% nuclear reaction of tumor cells.

gnosis:
breast 11 o'clock suspicious hypoechoic lesion, Tru-cut biopsy:
rmediate grade DCIS (Solid & cribriform), with microinvasive focus.
(Done in microinvasive focus):
: Negative (score 0/8).
: Positive (score 3/8).

Verified by :
Dr.Toka Medhat

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Eman Gomaa Monged Mohamed	Date : 24/05/2025
Exam : C.T CHEST WITH CONTRAST	Age : 40 Year(s)
Medical ID: 131/2025	Pat. ID: 99779

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of breast cancer.

I Chest:

- Left breast UIQ area of asymmetrical enhancing density measuring 2.5 X 5.5 cm.
- Prominent left axillary level I and II lymph node reachig 9 X 14 mm.
- No sizable pulmonary nodules, masses, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenaution. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Tiny L3 lucent focus.

Opinion:

Reported case of breast cancer revealed:

- **No sizable pulmonary or hepatic metastatic deposits.**

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient ID: 79673	Patient Name English: Eiman Gumaah Monged
Age: ٣٠ سنة	2025/03/20 10:22:56 AM :Report Date

Women's Health Outreach Program

Name: iman goma monged Date: 20-3-2025

Bilateral Full field Digital Mammography

Clinical summary: 40 ys old lady with left breast palpable lump of 3 months duration . stop lactating from 6 month

Technique

Medio-lateral oblique and CC views , High resolution ultrasound

Detailed Findings :

Left breast

- Scattered fibroglandular densities (ACR density b).
- UIO asymmetrical density in area about 5x4cm, away from nipple by 4.5cm.
- No distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated a corresponding irregular hypoechoic area with irregular margin seen at 11:00 middle zone about 4.5 cm from the nipple and 3:6 mm from the overlying skin, it seen measuring 4x 2.3 x 3 cm in average dimensions , minimal vascularity on color Doppler interrogation . with prominent ducts ??? ductal extension
- Normal skin , areolar complex.
- Multiple pathological axillary LNs seen at level I , II and infra clavicular showing thickened non uniform cortices reaching 8 mm , effaced hila ,at level I measures 15x7mm ,at level II measures 7x5mm and infra calvicualr measures 10x6mm

Right breast

- Scattered fibroglandular densities (ACR density b).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound confirmed no suspicious masses or cystic lesions .
- Mild retroareolar prominent ducts
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Left breast (BIRADS 4c) UIQ asymmetry with underlying irregular heterogeneous area for histopathological verification & biopsy .
- Right breast (BIRADS 2): prominent retroareolarducts .

Much obliged

Dr. Marwa M.Hassan

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient ID: 86765	Patient Name English: Eiman Gumaah Monged Mohamed
Age: ٣٠ سنة, ٥ شهر	2025/08/17 10:41:32 AM :Report Date

Right Ultrasound guided clip insertion

Indication:

Right breast:

- Under complete aseptic condition & ultrasound guidance one clip insertion was done for left breast UIQ malignant lesion lesion at 11 o'clock, the clip was inserted at the center of the lesion and this was confirmed by mammographic CC & ML

Best Regards,

Dr. Heba Azzam, MD



Ultrasound
[[DOCTYPE:ULTRASOUND]]

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION SYSTEM AND DIGITAL
TRANSFORMATION

الهيئة العامة للتامين الصحى
الادارة المركزية للنظم المعلومات و التحول الرقمى

رسوم الاستعلام 20 جنيه

جهه الاصدار :فرع القاهرة

تاريخ الاصدار :2025-3-20

الى من يهمه الامر .....

الهيئه العامة للتامين الصحى تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التامين الاجتماعى رقم 79 لسنه 1975 المعدل بالقانون 25 لسنه 1977 "القوى العامله والمعاشات والارامل " و كذلك قانون 99 لسنه 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنه 2012 " الاطفال دون السن المدرسى" وقانون رقم 23 لسنه 2012 الخاص بالمرأة المعيله .

وطبقا لقاعدة البيانات المرسلة من التأمينات الاجتماعيه

وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعى

والبحث تبين أن

السيد (ة) ايمان جمعه منجد محمد بطاقة رقم 28502210102583

تليفون رقم: 00

وطبقا الى خطاب من التأمينات الإجتماعية يفيد عدم الأنتفاع
يعتبر من الفئات غير المنتفعه بالتامين الصحى صدر هذا الخطاب دون ادنى مسئولية على الهيئة العامة للتأمين الصحى حال خضوع المواطن لقانون التأمين الصحى الشامل

وشكرا

تاريخ الطباعه :2025-3-20

الموظف المختص

اسلام عبد الحميد

افادة مركز نظم المعلومات
يعتمد،،

مسلسل 1608287

الخطاب سارى لمدة ثلاثة أشهر من تاريخ الإصدار الا فى حاله تغير حاله المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو وزارة التضامن الاجتماعى أو وزارة التربية و التعليم

الادارة المركزية لنظم المعلومات و التحول الرقمى

Non-HIO Letter
[[DOCTYPE:NON_HIO_LETTER]]

Nuclear Medicine Unit

Patient Name: Eiman Gomaa Monged *Date: 29/7/2025*

^{99m}Tc-MDP BONE SCAN

Desired information: *skeletal surveying to detect multiple lesions.*

Referring physician: Prof.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No defenite active osseous metastatic lesions could be seen all over the surveyed body .
- Abnormal heterogenous radiotracer uptake is seen at the dorso-lumbar spine , , likely of spondylo degenerative changes .
- Osteoarthritic changes are seen at both shoulders , both knees .
- Both kidneys are normally seen.

Conclusion:

- No definite active osseous metastatic lesions could be seen al over the surveyed skeleton .

Nuclear Medicine Unit

DR . Shorien Mahdy. MD

DR/ Mohamed Emam

Bone Scan
[[DOCTYPE:BONE_SCAN]]

* Serialling showed a tumor bed site 9x7x3 cm, located 3 from the least deep margin.
* Axillary fat measured 8x7x2 cm, entangling 11 lymph nodes, the largest measured 2.5x2x1 cm.
* 15 Blocks:
1:9- T.B.S., 10- N&A., 11- Deep margin., 12:15- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic Examination:
* Histological type: No residual viable remnants of invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.
* Nipple & areola: Free.
* Deep margin: Free.
* Axillary lymph nodes: Eleven lymph nodes free of metastatic deposits (0/11).
* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

referred To PORT,
She recived 40 Gy/ 15 #
ē good Tolerance
start 5-4-2026
End 29-4-2026
for your kind care

د. ابراهيم ماجد
نائب علاج الأورام و الطب النووي

شارع جوهر القائد، الدراسة، القاهرة تليفون ٢٥١١١٩٨١ (٢+) - ٢٥٠٦٣٥٦٠ (٢+)
Gawhar Al-kaed St, Al-Darrasa, Cairo Egypt phone& Fax (+2) 25111981 - (+2) 25063560

* Serialling showed a tumor bed site 9x7x3 cm, located 3 from the least deep margin.
* Axillary fat measured 8x7x2 cm, entangling 11 lymph nodes, the largest measured 2.5x2x1 cm.
* 15 Blocks:
1:9- T.B.S., 10- N&A., 11- Deep margin., 12:15- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic Examination:
* Histological type: No residual viable remnants of invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.
* Nipple & areola: Free.
* Deep margin: Free.
* Axillary lymph nodes: Eleven lymph nodes free of metastatic deposits (0/11).
* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

referred To PORT,
She recived 40 Gy/ 15 #
ē good Tolerance
start 5-4-2026
End 29-4-2026
for your kind care

د. ابراهيم ماجد
نائب علاج الأورام و الطب النووي

شارع جوهر القائد، الدراسة، القاهرة تليفون ٢٥١١١٩٨١ (٢+) - ٢٥٠٦٣٥٦٠ (٢+)
Gawhar Al-kaed St, Al-Darrasa, Cairo Egypt phone& Fax (+2) 25111981 - (+2) 25063560

* Serialling showed a tumor bed site 9x7x3 cm, located 3 from the least deep margin.
* Axillary fat measured 8x7x2 cm, entangling 11 lymph nodes, the largest measured 2.5x2x1 cm.
* 15 Blocks:
1:9- T.B.S., 10- N&A., 11- Deep margin., 12:15- ALNs.
- Gross was done by; Dr. Heba El-Shafeey.

Microscopic Examination:
* Histological type: No residual viable remnants of invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.
* Nipple & areola: Free.
* Deep margin: Free.
* Axillary lymph nodes: Eleven lymph nodes free of metastatic deposits (0/11).
* Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of invasive tumor.
- Ductal carcinoma in situ: Absent.
- Nipple & areola: Free.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

referred To PORT,
She recived 40 Gy/ 15 #
ē good Tolerance
start 5-4-2026
End 29-4-2026
for your kind care

د. ابراهيم ماجد
نائب علاج الأورام و الطب النووي

شارع جوهر القائد، الدراسة، القاهرة تليفون ٢٥١١١٩٨١ (٢+) - ٢٥٠٦٣٥٦٠ (٢+)
Gawhar Al-kaed St, Al-Darrasa, Cairo Egypt phone& Fax (+2) 25111981 - (+2) 25063560

Patient ID: 79673	Patient Name English: Eiman Gumaah Monged
Age: ٤٠ سنة	2025/03/20 10:22:56 AM :Report Date

Women's Health Outreach Program

Name: iman goma monged **Date:** 20-3-2025

Bilateral Full field Digital Mammography

Clinical summary: 40 ys old lady with left breast palpable lump of 3 months duration . stop lactating from 6 month

Technique

Medio-lateral oblique and CC views , High resolution ultrasound

Detailed Findings :

Left breast

- Scattered fibroglandular densities (ACR density b).
- UIO asymmetrical density in area about 5x4cm, away from nipple by 4.5cm.
- No distortion or suspicious clusters of microcalcifications.
- **Ultrasound demonstrated a corresponding irregular hypoechoic area with irregular margin seen at 11:00 middle zone about 4.5 cm from the nipple and 3:6 mm from the overlying skin, it seen measuring 4x 2.3 x 3 cm in average dimensions , minimal vascularity on color Doppler interrogation . with prominent ducts ??? ductal extension**
- Normal skin , areolar complex.
- **Multiple pathological axillary LNs seen at level I , II and infra clavicular showing thickened non uniform cortices reaching 8 mm , effaced hila ,at level I measures 15x7mm ,at level II measures 7x5mm and infra calvicualr measures 10x6mm**

Right breast

- Scattered fibroglandular densities (ACR density b).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** confirmed no suspicious masses or cystic lesions .
- **Mild retroareolar prominent ducts**
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Left breast (BIRADS 4c) UIQ asymmetry with underlying irregular heterogeneous area for histopathological verification & biopsy .**
- **Right breast** (BIRADS 2): prominent retroareolarducts .

Much obliged

Dr. Marwa M.Hassan

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient's Name: **Iman Jum'ah Munjid Mohammed**

Date: **2026-09-15**

Referred by: Prof.Dr. **no doctor**

Hospital: **Goustave Roussey International Egypt**

Right Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Post left MRM & ALAND, routine follow up.

Findings:

Right breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 1): normal study.

Clear left mastectomy bed and axilla.

Recommendation:-

For annual follow up.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Mammogram
[[DOCTYPE:MAMMOGRAM]]